



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : هاله حسن العزب صالح
المحافظة : الغربية
الحى / المدينة : السنطة
العنوان : -
رقم البطاقة : 27009231601723 رقم التليفون : 01028764724
العمر (تاريخ الميلاد) : 23-09-1970 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 22-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعة مقطعية كتف مع صبغة [10851] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- / د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هاله حسن العزب صالح National ID: 27009231601723	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of the shoulder.
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

PATIENT NAME: HALA HASSAN ELAZAB SALEH

GENDER: Female

DATE : 12-Jan-2026 03:15 PM

PATIENT ID: 2021160162
AGE: 55 y
ORDER: 3723566,3723567,3724112

FULL FIELD DIGITAL MAMMOGRAPHY

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Targeted ultrasound

FINDINGS:

There are scattered areas of fibroglandular density (ACR b).

LEFT BREAST:

- Rather stationary UOQ spiculated dense mass, measuring 6.5x6 cm in diameter 15 mm from skin and 5.8cm from the nipple, targeted ultrasound revealed corresponding 2:00 spiculated mass with internal vascularity on color Doppler.
- LOQ small irregular nodule is seen more appreciated in the current study, being 6.5cm inferior to the aforementioned mass and 7cm from nipple, targeted ultrasound revealed corresponding small irregular hypoechoic nodule surrounded with hyperechoic desmoplastic reaction, located at 3:00, measuring about 0.8x0.6cm in diameter.
- Mild areolar and paraareolar skin thickening are still noted with subtle nipple retraction
- No pathologically enlarged axillary lymph nodes.

RIGHT BREAST:

- No spiculated mass lesions or grouped microcalcifications.
- Targeted Ultrasound of outer quadrants revealed no suspicious lesions.
- Scattered round calcifications.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Rather stationary left breast UOQ pathologically proved malignant mass (BIRADS 6) along with LOQ newly appreciated small suspicious nodule.
- Right breast benign findings (BIRADS 2).

PATIENT: HALA HASSAN ELAZAB SALEH, 2021160162, Female, 55 y

Report version 1, 12-Jan-2026 05:19 PM

MRN:	202160162	Patient Name:	HALA HASSAN ELAZAB SALEH
D.O.B:	23/09/1970	IC No.:	27009231601723
Sex:	Female	Location:	External Clinic

SURGICAL PATHOLOGY
Biopsy Small - (1 Site)

Specimen Id	2107120243	Collection Date	12/07/2021 01:05 PM
Accession No	SUR21-5266	Received Date	12/07/2021 01:06 PM
Sample Type	Tissue	Doctor	Abd ALLAH Mohamed Gamil

Clinical Information

Lt. Breast, UOQ mass.
Gross Picture
5 cores, 0.5-1.5 cm
Microscopic Picture
A case of invasive duct carcinoma, grade II. Stroma is desmoplastic with mild inflammatory cell reaction. There is focal DCIS component, I

Diagnostic Summary

Tumour type:
IDC, grade II

Laterality
Left

Specimen type:

Tru-cut biopsy

Location
Upper outer quadrant

Histological grade
Mitosis score (1-5):

2
Tubular formation score (1-5):

2
Nuclear grade (1-3):

2
Histological grade (1-3):

Peritumoural lymphovascular invasion
Absent

Diagnosis

Lt. Breast, tru-cut biopsy:

Invasive duct carcinoma, grade II.

Comments

ER, PR and Her 2 are ordered.

Biopsy reporting category

B5b= Invasive cancer

Signature

MONA MOHAMED MAMDOH

Co-signed by:

Medical Reports

Patient Information

MRN	2021160162	Name	HALA H/ASSAN ELAZAB SALEH	
ID Type	الرقم القومي	ID Number	27009231601723	
Gender	FEMALE	Nationality	EGYPTIAN	
Age	55Year(s)			
Written Date	26/05/2025	5:16 pm	Visit Date	19/05/2025
Diagnosis	[P](C50.9) Malignant neoplasm of breast, unspecified part			

Report and Recommendation

Postmenopausal 54 ys with MBC (bone)T4 N2 M1 ,ER:8/8 ,PR 5/8 , Her2: -ve received TAMI/ zoladex from 8/2021 10/2022--> PD--> AC x6--> AI /Zometa 2/2023 Planned for CDK4,6 Inhibitors but was weaved till progression CT CAP 3/2024: stationary course since the last study dated 16/10/2023. Continue AI & Zometa every 3 months, pt reported spotting so Zoladex added again since 5/2/2024 CT CAP: 15/5/2025

stationary course with no newly developed lesions.

The examined skeleton shows stationary course as regard:

- Widespread sclerotic osseous lesions.
- Collapsed D3 vertebra seen indenting the cord (previously irradiated)
- Vertical cleavage of D10 vertebral body.

Bone scan:

partial regression of metastatic deposits

Written By

AMIRA Abd-Elmoniem DARWISH

Approved By

AMIRA Abd-Elmoniem DARWISH

Medical Director



09/22/2026

1000

MDT FORM

Name:	مكة صبح الجرب صبح	MRN:	13137
Age:	56 Y	ID:	27009231601723
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC GII

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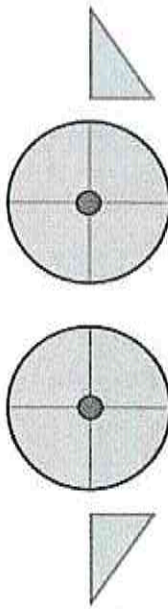
Figure 1

<u>ER:</u>	8/8	<u>PR:</u>	6/8	<u>HER2:</u>	0	<u>KI67:</u>	15
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Sonomammography:

Meets Workup:



TNM

History:

55 years old female patient, pre menopausal received tam + zoladex from 8/2021 to 11/2022 -> PD. received AC6 with regression then kept on maintenance AI + zometa from 2/2023 to 1/2026.

ct cap 1/2026 + bone scan >> SD but clinically progressed with severe back

24 pain

MRI cervical > multiple vertebral osseous lesion likely mets with c5+d3 cord compression

compression

received palliative RTH to C4>D3

Recommendations:

Recommendations:

shifted to faslodex + ibranse

ct cap + MRI whole spine + b

MDT Outcome & Plan

shifted to faslodex + ibrance

ct cap + MRI whole spine + b

ct cap + MRI whole spine + bone scan as base line in our center

SIGNATURE

	MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
مستوى المعرفة						
مستوى التطبيق						
مستوى التقييم						

15/10/2015

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناتيونال - مصر -معمل الباثولوجي

Patient Name	ملاء حسن العرب	Case No.	357643
Sex/Age	Female 55 Y	Patient No.	25798
Registered Date	12/8/2026 11:34:17 AM	Referred By Dr.	Out Patient
Validation Date	25/8/2026 01:45:54 PM	Branch	Gustave Roussy
Reporting Date	14/9/2026 10:59:35 AM	Pathology No.	SR-930/26

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Block revision.

Gross Examination:

- One paraffin block labeled by (4828/23) was received.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 1).
- DCIS: Absent.
- Microcalcification: Present.
- Angioinvasion: Absent.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (score 0+).
- Ki67%: 15%.

Conclusion:

Left breast cancer, Block revision:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Ultra low (score 0+).
- Ki67%: 15%.

شقة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt



Gustave Roussy International-Egypt- Pathology Lab
جوستاف رويسى انترناشيونال - مصر -معمل الباثولوجى



Patient Name	مالة حسن الزرب	Case No.	357643
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****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Choneim, MD.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mhar El Qadima - Cairo, Egypt

09/22/2026

MDT FORM

Name:	هالة حسن الغرب صالحي	MRN:	13137
Age:	56 Y	ID:	27009231601723
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC GII

ER: 8/8 **PR:** 6/8 **HER2:** 0 **KI67:** 15

Sonomammography:

Mets Workup:

History:

55 years old female patient, pre menopausal received tam + zoladex from 8/2021 to 11/2022 >> PD. received AC*6 with regression then kept on maintenance AI + zometa from 2/2023 to 1/2026.

ct cap 1/2026 + bone scan >> SD but clinically progressed with severe back pain

MRI cervical >> multiple vertebral osseous lesion likely mets with c5+d3 cord compression

recommendation for faslodex + cdk4/6 inhibitor but not received received palliative RTH to C4>D3

Recommendations:

shifted to faslodex + ibrance

ct cap + MRI whole spine + bone scan as base line in our center

MDT Outcome & Plan

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ct cap + MRI whole spine + bone scan as base line in our center

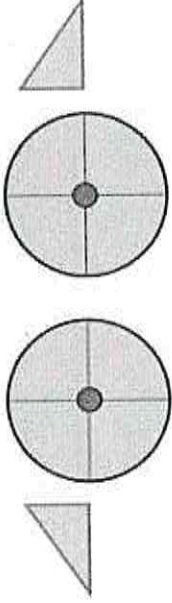
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

د. محمد كركاشي

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TNM

09/22/2026

MDT FORM

Name:	علي محمد علي	MRN:	13137
Age:	56 Y	ID:	27009231601723
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC GII

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<u>ER:</u>	8/8	<u>PR:</u>	6/8	<u>HER2:</u>	0	<u>KI67:</u>	15
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Meets Workup:

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ct cap 1/2026 + bone scan >> SD but clinically progressed with severe back

TNM

pain
MRI cervical >> multiple vertebral osseous lesion likely mets with C5+D3 cord compression

received palliative RTH to C4>D3

Recommendations:

Recommendations:

shifted to faslodex + ibuprofen

ct cap + MRI whole spine + bone scan as base line in our center

MDT Outcome & Plan

shifted to faslodex + ibranon

ct cap + MRI whole spine + bone scan as base line in our center

SIGNATURE

2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	29	30	31	32	33	34	35	36	37	38	39	40	41	42	43	44	45	46	47	48	49	50	51	52	53	54	55	56	57	58	59	60	61	62	63	64	65	66	67	68	69	70	71	72	73	74	75	76	77	78	79	80	81	82	83	84	85	86	87	88	89	90	91	92	93	94	95	96	97	98	99	100	101	102	103	104	105	106	107	108	109	110	111	112	113	114	115	116	117	118	119	120	121	122	123	124	125	126	127	128	129	130	131	132	133	134	135	136	137	138	139	140	141	142	143	144	145	146	147	148	149	150	151	152	153	154	155	156	157	158	159	160	161	162	163	164	165	166	167	168	169	170	171	172	173	174	175	176	177	178	179	180	181	182	183	184	185	186	187	188	189	190	191	192	193	194	195	196	197	198	199	200	201	202	203	204	205	206	207	208	209	210	211	212	213	214	215	216	217	218	219	220	221	222	223	224	225	226	227	228	229	230	231	232	233	234	235	236	237	238	239	240	241	242	243	244	245	246	247	248	249	250	251	252	253	254	255	256	257	258	259	260	261	262	263	264	265	266	267	268	269	270	271	272	273	274	275	276	277	278	279	280	281	282	283	284	285	286	287	288	289	290	291	292	293	294	295	296	297	298	299	300	301	302	303	304	305	306	307	308	309	310	311	312	313	314	315	316	317	318	319	320	321	322	323	324	325	326	327	328	329	330	331	332	333	334	335	336	337	338	339	340	341	342	343	344	345	346	347	348	349	350	351	352	353	354	355	356	357	358	359	360	361	362	363	364	365	366	367	368	369	370	371	372	373	374	375	376	377	378	379	380	381	382	383	384	385	386	387	388	389	390	391	392	393	394	395	396	397	398	399	400	401	402	403	404	405	406	407	408	409	410	411	412	413	414	415	416	417	418	419	420	421	422	423	424	425	426	427	428	429	430	431	432	433	434	435	436	437	438	439	440	441	442	443	444	445	446	447	448	449	450	451	452	453	454	455	456	457	458	459	460	461	462	463	464	465	466	467
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15/10/2015



د. يوسف
رئيس
الفرع
أ- ٤٢

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

المنشأة المصرية
للطب

09/22/2026

MDT FORM

Name:	هالة حسن الغرب صالحي	MRN:	13137
Age:	56 Y	ID:	27009231601723
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

R

BX:	IDC GII
ER:	8/8
PR:	6/8
HER2:	0
KI67:	15

Sonomammography:

Mets Workup:

History:

55 years old female patient, pre menopausal received tam + zoladex from 8/2021 to 11/2022 >> PD. received AC*6 with regression then kept on maintenance AI + zometa from 2/2023 to 1/2026.

ct cap 1/2026 + bone scan >> SD but clinically progressed with severe back pain

MRI cervical >> multiple vertebral osseous lesion likely mets with c5+d3 cord compression

recommendation for faslodex + cdk4/6 inhibitor but not received received palliative RTH to C4>D3

Recommendations:

shifted to faslodex + ibrance

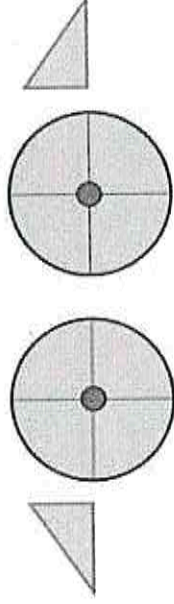
ct cap + MRI whole spine + bone scan as base line in our center

MDT Outcome & Plan

shifted to faslodex + ibrance

ct cap + MRI whole spine + bone scan as base line in our center

TNM



L

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد كركاش					

إسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،

إيماء الطالب المقدم من بشأن تقرير طبي للمريض: حالة

بقسم رقم طبي

نشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

رأى السرطان من السرطان و السرطان السرطان

حالة السرطان السرطان السرطان

.....

تحريراً في

الوقت: التاريخ: / /

الطبيب المعالج

يقتط ،،

17/11/21

بشأن تقرير طبي للمريض: صالح محمد

تاريخ وساعة الدخول تاريخ وساعة الخروج.

Chlorine gas 1000 cc

1. $\frac{1}{2} \frac{d}{dt} \left(\frac{1}{2} \frac{d^2}{dt^2} \right)$

...

03/05/2020

الطبيب المعالج

Page 1 of 1

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،

إيماء الطلب المقدم من

بقسم

نشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول تاريخ وساعة الخروج

ويط الكشف والفحوصات والمتابعة وجد أنه:

وزيد محمد طه ووليد طه

علاء الدين

2016/09

تحريراً في

الوقت: / / التاريخ:

الطبيب المعالج

يختم

اسم المريض ثلاثي:.....
رقم الملف:.....

تقرير طبي

السيد/

تحية طيبة وبعد،،

إيماء الطالب المقدم من

بقسم

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

ويط الكشف والفحوصات والمتابعة وجد أنه:

٢١) السيد حسن وليد السكاك
صاحبنا هادي خان فاروق
+ بالبريد +

تحريراً في

الوقت: التاريخ: / /

الطبيب المعالج

يتمتع

Patient Name	ملاء حسن العرب	Case No.	357643
Sex/Age	Female 55 Y	Patient No.	25798
Registered Date	12/8/2026 11:34:17 AM	Referred By Dr.	Out Patient
Validation Date	25/8/2026 01:45:54 PM	Branch	Gustave Roussy
Reporting Date	14/9/2026 10:59:35 AM	Pathology No.	SR-930/26

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Block revision.

Gross Examination:

- One paraffin block labeled by (4828/23) was received.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
 - ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 1).
- DCIS: Absent.
- Microcalcification: Present.
- Angioinvasion: Absent.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (score 0+).
- Ki67%: 15%.

Conclusion:

Left breast cancer, Block revision:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Ultra low (score 0+).
- Ki67%: 15%.



Patient Name	مالة حسن الزرب	Case No.	357643
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****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Choneim, MD.


غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Nhar El Qadima - Cairo, Egypt



د. يوسف
رئيس
الفرع
٨١ - ٤٢

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

المنشأة المصرية
مصر

09/22/2026

MDT FORM

Name:	هالة حسن الغرب صالحي	MRN:	13137
Age:	56 Y	ID:	27009231601723
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:	IDC GII						
ER:	8/8	PR:	6/8	HER2:	0	KI67:	15

Sonomammography:

Mets Workup:

History:

55 years old female patient, pre menopausal received tam + zoladex from 8/2021 to 11/2022 >> PD. received AC*6 with regression then kept on maintenance AI + zometa from 2/2023 to 1/2026.

ct cap 1/2026 + bone scan >> SD but clinically progressed with severe back pain

MRI cervical >> multiple vertebral osseous lesion likely mets with c5+d3 cord compression

recommendation for faslodex + cdk4/6 inhibitor but not received received palliative RTH to C4>D3

Recommendations:

shifted to faslodex + ibrance

ct cap + MRI whole spine + bone scan as base line in our center

MDT Outcome & Plan

shifted to faslodex + ibrance

ct cap + MRI whole spine + bone scan as base line in our center

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد كركاشي					

09/22/2026

MDT FORM

Name:	ملكة حسن العرب صالح	MRN:	13137
Age:	56 Y	ID:	27009231601723
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC GII **R**

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T N M

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T N M

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T N M

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T N M

Recommendations:
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T N M

MDT Outcome & Plan

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

2 منصف كرام
Lynx

SIGNATURE

إسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،

إيماء الطالب المقدم من بشأن تقرير طبي للمريض: حالة

بقسم رقم طبي

نشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

رأى السرطان من السرطان و السرطان السرطان

حالة السرطان السرطان السرطان

د د د

تحريراً في

الوقت: التاريخ: / /

الطبيب المعالج

يقتط ،،

17/11/21

بشأن تقرير طبي للمريض: صالح محمد

نتسرف بان نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج.

Chlorine gas 1000 cc

$\frac{1}{\sqrt{2}} \begin{pmatrix} 1 & i \\ 0 & 1 \end{pmatrix}$

...

03/03/2020

الوقت: التاريخ: / /

بیعت

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

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بقسم

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علاء الدين

2016/09

تحريراً في

الوقت: / / التاريخ:

الطبيب المعالج

يعتمد

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،

إيماء الطالب المقدم من بشأن تقرير طبي للمريض: حارس عراب
بقسم رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :
تاريخ وساعة الدخول تاريخ وساعة الخروج

ويط الكشف والفحوصات والمتابعة وجد أنه:

(٢١) السيد حسن ولد المرحوم السيد خالد
صاحبنا هطانا كارولست
+ بالبريد + ماله ماله

تحريراً في

الوقت: التاريخ: ١ / ١

الطبيب المعالج

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